



Situation Report

Lebanon Crisis

23 June - 7 July 2026

Highlights

The humanitarian situation in Lebanon remains fragile despite a reduction in hostilities, with significant and persistent humanitarian, protection, and early recovery needs. Daily airstrikes and military operations, particularly in southern Lebanon, continue to pose risks to civilians and limit safe returns, highlighting the potential for renewed escalation despite ongoing diplomatic efforts. [Since 2 March 2026, a reported 4,304 people have been killed and 12,203 have been injured.](#)

Women and girls are bearing the brunt of the crisis, facing heightened protection and gender-based violence (GBV) risks, disrupted health services, including for maternal health, and reduced access to safe and dignified support.

Since the onset of the crisis, UNFPA has reached over 150,000 displaced people with sexual and reproductive health (SRH) and GBV services. These services have been delivered through support to 55 health facilities, nine mobile medical units, 24 mobile GBV teams, and 19 women and girls' safe spaces (WGSSs), serving collective shelters and host communities.



1.14 million

Total people affected¹



390,000

Women of reproductive age²



16,000

Estimated pregnant women²



150,000

People targeted with SRH services



300,000

People targeted with GBV programmes

¹ [Lebanon: Flash Update #40 - Escalation of hostilities in Lebanon. OCHA](#)

² Estimated figures are based on the Minimum Initial Services Package (MISP) for Sexual and Reproductive Health in Humanitarian Settings calculator. This includes the original number 1.14 million affected people, still displaced persons and returnees.

Situation Overview

An estimated [646,107 people in Lebanon have begun returning to their areas of origin](#), with population movements increasing by 23 per cent in the past two weeks. The largest share of returnees has been reported in South and Nabatieh governorates, where the conflict has caused extensive damage to housing, infrastructure, and essential services. Despite the increase in returns, movements remain cautious and uneven. Many households continue to undertake temporary “go-and-see” visits to assess levels of destruction, security conditions, and service availability before returning permanently.

Displacement levels continue to decline, with the proportion of displaced persons residing in collective shelters decreasing from 11 per cent to 9 per cent over the past two weeks. The number of registered collective sites has reduced from over 600 to [430 and the collective-site population has reduced to less than half from over 100,000 to 47,143 individuals since 25 June](#). As of 1 July 2026, some [499,784 internally displaced persons have been recorded across Lebanon](#). New shelters are being established in Nabatieh and South governorates to accommodate individuals undertaking “go-and-see” visits prior to return, as well as households whose homes have been damaged, destroyed, or remain inaccessible. Many displaced households continue to face uncertainty regarding future accommodation arrangements, including a need for secondary shelter solutions and cash support for rent.

Women and girls in collective shelters, temporary accommodation, and return areas continue to face protection risks. Despite reduced shelter occupancy, key vulnerabilities persist, including limited privacy, unsafe or inadequately segregated water and sanitation facilities, weak access to confidential GBV services, and rising rental costs and eviction threats. Female-headed households, adolescent girls, pregnant and lactating women, older persons, and persons with disabilities encounter specific barriers to safety and services. Sustained outreach, survivor-centred GBV response, functional referral pathways, and risk mitigation measures are essential across shelters and communities hosting displaced and returning families.

Health services remain severely strained. [Three hospitals and 36 primary health centres \(PHCs\) remain closed, while 17 hospitals are damaged](#). Although some hospitals in southern Lebanon have resumed essential maternity services, access to safe maternal and newborn care remains limited, particularly in Bent Jbeil and Nabatieh districts, where maternity services have not been fully restored and emergency care has been prioritized. Returns to areas with widespread damage, insecurity, and limited services are further disrupting service provision, mobile service deployment, and continuity of care. Sustained support for primary healthcare, mobile outreach, obstetric and newborn care, and GBV services is essential, particularly in southern Lebanon, Saïda, Beirut, and Mount Lebanon.

UNFPA estimates that 390,000 women of childbearing age are among the displaced and returnee population, of whom approximately 16,000 are pregnant, and around 1,800 women are expected to give birth every month. Pregnant women are returning to areas with significant damage to health infrastructure, in particular for maternal healthcare, underscoring the continued need for life-saving SRH supplies and services.

The operating environment remains challenging. Access constraints and funding shortfalls continue to limit the response. Damage to infrastructure and service networks, together with persisting insecurity in parts of South and Nabatieh governorates, continue to affect humanitarian operations and early recovery efforts.

UNFPA Response

Sexual and reproductive health: Since 2 March, UNFPA has delivered SRH services to more than 38,500 women and girls residing in collective shelters and with host communities. Support is being provided through PHCs, mobile medical units, and a network of midwives and social workers. In addition, 506 baby kits³ were distributed to displaced pregnant women in their third trimester and to new mothers. During the reporting period, cash assistance was provided to 81 pregnant and postpartum mothers to cover essential maternity care costs.

UNFPA supported nearly 1,600 pregnant women to access facility based deliveries. Four shipments of inter-agency reproductive health (IARH) kits delivered via the European Union Humanitarian airbridge, were dispatched to hospitals and PHCs via the Ministry of Public Health, covering the reproductive health needs of approximately 30,000 women of childbearing age.

Gender-based violence: Since March 2026, more than 45,000 individuals have been reached with GBV services and information both outside and within 155 collective shelters. Among them, over 6,100 displaced persons benefited from specialized GBV services, including case management, psychosocial support, recurrent cash assistance and referrals. Nearly 38,000 individuals were reached with GBV awareness messages including on protection from sexual exploitation and abuse.

More than 28,700 women and girls of reproductive age received dignity kits.⁴ Additionally, dignity kits were dispatched via World Food Programme convoys to Hasbaya in southern Lebanon.

In partnership with the Ministry of Social Affairs (MoSA) and UN agencies, training for 350 social workers affiliated with the Ministry on GBV and protection principles is currently ongoing.

In collaboration with the MoSA, multi-purpose cash assistance was provided to 956 individuals under the [Shock Response Safety Net initiative](#). This initiative aims to support displaced and conflict-affected households by helping them meet their most urgent needs. This support also helps minimize negative coping mechanisms during a period of heightened vulnerability.



³ Baby kits contain a blanket, baby clothes, baby hygiene material, diapers, towels, baby rash cream, and breastfeeding pads, in addition to other essentials.

⁴ Dignity kits contain a three-month supply of essential items to maintain hygiene and health. They include sanitary pads, soap, a torch, socks, underwear, toothbrush and toothpaste, and a leaflet with key messages on GBV, the prevention of sexual abuse and exploitation, psychosocial support, and other available services.

Results Snapshot (2 March - 6 July 2026)



38,550

People reached with SRH services



55

Health facilities supported



45,248

People reached with GBV prevention, mitigation and response activities



19

Safe spaces for women and girls supported



28,775

Dignity kits distributed to women and adolescent girls



9

Mobile medical units deployed



1,727

People reached with humanitarian cash and voucher assistance for SRH and GBV



24

GBV mobile teams deployed

Coordination Mechanisms

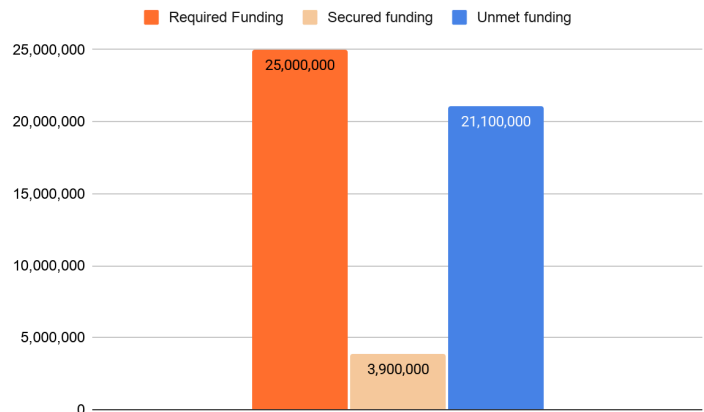
GBV Sub-Sector Working Group: Co-led by UNFPA, the UN High Commissioner for Refugees (UNHCR), and Himaya Daeem Aataa, a national women-led organization, and in collaboration with MoSA and the Disaster Risk Management Unit, the GBV Working Group continues to coordinate the GBV response for displaced persons. Recent advocacy efforts included briefings to the Humanitarian Country Team on key GBV risks, emerging trends, and funding gaps, as well as briefings to the UN Resident/Humanitarian Coordinator on prioritized interventions under the Lebanon Humanitarian Fund in collaboration with the Protection and Child Protection sectors.

The Working Group reviewed operational reach via the protection sector displaced persons dashboard, highlighting that since March 2026, GBV interventions have been implemented across 407 collective shelters, reaching approximately 23,300 displaced persons.

SRH Working Group: Co-chaired by the Ministry of Public Health (MoPH) and UNFPA, the SRH Working Group continues to actively coordinate the SRH response to the crisis. The Working Group is prioritizing areas with high returnee numbers to raise awareness of available SRH services and strengthen referrals to operational health facilities. Together with the MoPH, the Working Group reviewed the [2025 Maternal and Neonatal Vital Data Observatory](#), which showed declining birth rates among Syrian refugees, suggesting that more women are delivering in Syria before re-entering Lebanon. The Working Group is now coordinating with the MoPH and outreach partners to strengthen monitoring of out-of-hospital deliveries in Lebanon, especially among Syrian women.

Funding Status

UNFPA launched a revised [Flash Appeal](#) in June, aligned with the revised [Inter-Agency Lebanon Flash Appeal](#) appealing for US \$25 million to meet the urgent needs of 450,000 people from March to August 2026. To date, only 16 per cent of the required funding has been secured.



Disclaimer: Funding available is based on cash funding rolled over from previous years, and transfers from/to other UNFPA departments. It does not include funds from agreements that have been signed but not yet received.

"When hospitals close and healthcare collapses, it is pregnant women who pay the price because women do not stop giving birth in the midst of a conflict. No woman should have to risk her life — or her baby's life — to give birth. Women's and girls' health and rights are non-negotiable even in a crisis."

— Anandita Philipose, UNFPA Representative in Lebanon.

Current Donors

- Central Emergency Response Fund
- European Union Humanitarian Aid
- Government of Norway
- Swedish International Development Cooperation
- Korean International Cooperation Agency
- UNFPA Emergency Fund

For more information

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